REHAB SOLUTIONS
Spine & Orthopedic Physical Therapy

Rehab Solutions Physical Therapy
1103 E. Boxelder Rd., Suite U
Gillette, WY 82718 USA
Phone: 307-686-8177 Fax: 307-686-9484
Email: admin@rehabsolutionswy.com
Website: www.rehabsolutionswy.com

Neck Disability Index (NDI)

Intake/Progress Note/Discharge (Circle One)

Name: Vicky Pulliam Date of Birth: 12-06-85 Date: 2-7-2022

Instructions: This questionnaire has been designed to give the doctor information as to how your neck pain has affected your ability to manage everyday life. Please answer every section and mark in each section only **ONE** box that applies to you. We realized that you may consider that two of the statements in any one section relate to you, but please just mark the box that most closely describes your problem.

SECTION 1 – PAIN INTENSITY
- ☐ I have no pain at the moment.
- ☒ The pain is very mild at the moment.
- ☐ The pain is moderate at the moment.
- ☐ The pain is fairly severe at the moment.
- ☐ The pain is very severe at the moment.
- ☐ The pain is the worst imaginable at the moment.

SECTION 2 – PERSONAL CARE (Washing, Dressing, Etc.)
- ☐ I can look after myself normally, without causing extra pain.
- ☒ I can look after myself normally, but it causes extra pain.
- ☐ It is painful to look after myself and I am slow and careful.
- ☐ I need some help, but manage most of my personal care.
- ☐ I need help every day in most aspects of self-care.
- ☐ I do not get dressed; I wash with difficulty and stay in bed.

SECTION 3 – LIFTING
- ☐ I can lift heavy weights without extra pain.
- ☒ I can lift heavy weights, but it gives me extra pain.
- ☐ Pain prevents me from lifting heavy weights off the floor, but I can manage if they are conveniently positioned, for example, on a table.
- ☐ Pain prevents me from listing heavy weights off the floor, but I can manage light to medium weights if they are conveniently positioned.
- ☐ I can lift very light weights.
- ☐ I cannot lift or carry anything at all.

SECTION 4– HEADACHES
- ☐ I have no headaches at all.
- ☐ I have slight headaches that come infrequently.
- ☒ I have moderate headaches that come in frequently.
- ☐ I have moderate headaches that come frequently.
- ☐ I have severe headaches that come frequently.
- ☐ I have headaches almost all the time.

SECTION 5 – RECREATION
- ☐ I am able to engage in all my recreation activities, with no neck pain at all.
- ☒ I am able to engage in all my recreation activities, with some neck pain.
- ☐ I am able to engage in most, but not all, of my usual recreation activities because of pain in my neck.
- ☐ I can hardly do any recreation activities, because of pain in my neck.
- ☐ I am able to engage in a few of my usual recreation activities because of pain in my neck
- ☐ I cannot do any recreation activities.

SECTION 6 – READING
- ☐ I can read as much as I want with no pain in my neck.
- ☒ I can read as much as I want with slight pain in my neck.
- ☐ I can read as much as I want with moderate pain in my neck.
- ☐ I cannot read as much as I want because of moderate pain in my neck.
- ☐ I can hardly read at all because of severe pain in my neck.
- ☐ I cannot read at all.

SECTION 7 – WORK
- ☐ I can do as much work as I want to.
- ☒ I can do my usual work, but no more.
- ☐ I can do most of my usual work, but no more.
- ☐ I cannot do my usual work.
- ☐ I can hardly do any work at all
- ☐ I cannot do any work at all.

SECTION 8 – SLEEPING
- ☐ I have no trouble sleeping.
- ☒ My sleep is slightly disturbed (less than 1 hr sleepless)
- ☐ My sleep is mildly disturbed (1-2 hrs sleepless).
- ☐ My sleep is moderately disturbed (2-3 hrs sleepless).
- ☐ My sleep is greatly disturbed (3-5 hrs sleepless).
- ☐ My sleep is completely disturbed (5-7 hrs sleepless).

SECTION 9 – CONCENTRATION
- ☒ I can concentrate fully when I want to, with no difficulty.
- ☐ I can concentrate fully when I want to, with slight difficulty.
- ☐ I have a fair degree of difficulty in concentrating when I want to.
- ☐ I have a lot of difficulty in concentrating when I want to.
- ☐ I have a great deal of difficulty in concentrating when I want to.
- ☐ I cannot concentrate at all.

SECTION 10 – DRIVING
- ☒ I can drive my care without any neck pain.
- ☐ I can drive my car as long as I want, with slight pain in my neck.
- ☐ I can drive my car as long as I want, with moderate pain in my neck.
- ☐ I cannot drive my car as long as I want, because of moderate pain in my neck.
- ☐ I can hardly drive at all, because of severe pain in my neck.
- ☐ I cannot drive my car at all.

Total: _____ / 50

Today's Date: 2/7/2022

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Phone: 307-686-8177 Fax: 307-686-9484

Thank you for choosing Rehab Solutions. We are glad you are here! Fill this out to the best of your ability.

Name: Viktoriya Pulliam Date of Birth:

Occupation: Administrator

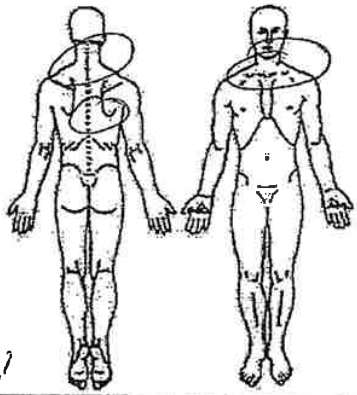
Hobbies: reading, gardening, hiking

Date of Injury: 11/23/2019 (Circle one) → Sudden or Gradual Onset

Referring Doctor: Sara Thornbury

Diagnosis: Spinal fusion

Surgery: Date: 11-4-2021



Please provide us with your **Past Medical History** below:

Surgical History:
Ectopic pregnancy April 2019
Cervical fusion Nov. 2021

Current Medications:
Escitalopram

Please mark your painful locations above.

Please place an X on the line below to indicate pain level and function.

Pain Level

0—1—2—3—4—5—6—7—8—9—10

Percent of Normal Function

0% 80% 100%

Please read and mark all that apply

Vision or Hearing Difficulties		Heart attack/surgery		Contagious Disease	
Cancer		Coronary Heart Disease		Infectious Disease	
Diabetes		High Blood Pressure		Asthma	
Dizziness/Fainting		Pacemaker		Pregnant	
Stroke/Mini Stroke		Blood Clots		Metal Implants	✓
Shortness of Breath		Seizures/Epilepsy		Smoker	
Chest Pain		Thyroid Dysfunction		Bowel and Bladder Problems	

Primary Complaint: Sore keloid, limited mobility

Goals for Physical Therapy: improve scar & visibility

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

CPT® Code	Direct Timed Codes	Units
GP:97140	Manual Therapy scar massage x 10 mins + instruction in self-scar massage and stretching scalene STM C1-2 GPA/UPAs grade 1-3 mid thoracic HVLA manipulation x 2 upper thoracic supine HVLA manipulation x 1 No cavitations. Pt tolerated manipulation well. Pt is appropriate for HVLA thrust manipulation techniques; has experienced them before with good response; no reported hx of osteoporosis or other factors affecting bone density.	2

CPT® Code	Untimed Codes	Units
GP:97162	PT Evaluation: Moderate Complexity	1

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Assessment

Assessment/Diagnosis: The patient presents for my initial evaluation s/p cervical fusion in response to 2 MVAs. She complains of 1. hypertrophic/keloid scar appearance and mobility, 2. new headaches, which seem to happen after every workout, and 3. new hypertension which occurs early in the morning and later in the evening.

The patient presents with cervical ROM limitations consistent with her cervical fusion, a hypertrophic scar with hypersensitivity and limited mobility, hypertension of 140/60 in the clinic today, and joint and soft tissue irritation throughout the cervical spine.

Physical therapy intervention will be able to address the limitations in ROM and scar mobility and appearance. Physical therapy intervention is also likely able to address the patient's headaches and hypertension, but further assessment is required to confirm. Additionally, the therapy team will collaborate with the patient's medical providers as appropriate.

We will continue to investigate the patient's headaches. Currently, the DDX includes:

1. Whiplash Associated Disorder
2. mTBI with features of autonomic dysfunction
3. Headache (By ICDH-3 criteria):
- 4.2.1 Probable primary exercise headache
- 4 Other primary headaches
- 10.3 Headache attributed to arterial hypertension
- 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy

We will also investigate the hypertension further with orthostatic and stress testing.

The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits and scar sensitivity.

Patient Education: Instruction in scar massage. Advised to continue use of silicone scar patches and moisturize with a gentle moisturizer.

Rehab Potential: Good

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP |
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep. |

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within |
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM. |
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck. |

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Daily Note / Billing Sheet

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989

Referring Physician(s): ACCESS, DIRECT MD

Surgery: (Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 1

Insurance Name

Date of Daily Note: 02/07/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury, Feb 2020 2nd MVA
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus
Date of Original Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
Current Complaints / Gains: CC: scar care
Secondary complaints: New HAs, new hypertension

Patient presents to therapy primarily to address the scar from her anterior cervical fusion. She also has a two week history of new headaches, which feel like her existing migraines, but occur after exercise. She also complains of new hypertension, especially in the morning and at night. Her upper cervical spine bothers her while trying to fall asleep since surgery.

Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Pain Location: neck over scar
Pain Scale: Worst: 3 **Best:** 0 **Current:** 0
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Is Patient A Tobacco User: No
Home Health Care: No
History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Mental Status/Cognitive Function Appears Impaired? No

Objective

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

Plan

Instructions: Progressing Patient Next Visit

Testing:
- OH, BP w/ valsalva, consider treadmill test w/ BP
- Vertebral artery positional testing

Tx:
- scar mobilization
- upper cervial mobilization, STM/MFR
- thoracic mob/manip

Dylan Ziehme

Dylan Ziehme
Initiated by Dylan Ziehme on February 8, 2022 at 2:26 am

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Electronically Signed by Aaron Proefrock on February 8, 2022 at 9:26 am

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Patient Outcomes

Report Date: 2/8/2022

VIKTORIYA PULLIAM 18 OF 100

Patient ID:	9610690	**Questionnaire Type:**	Initial Eval
Case Key:	42212797	**Area:**	Neck
Visit Date:	2/7/2022	**Clinician:**	Proefrock, Aaron
Entry Date:	2/8/2022	**Clinic:**	Rehab Solutions
ICD Code:	L910 - Hypertrophic scar	**Referral Source:**	ACCESS, DIRECT
Gender:	F		Commercial
Date of Birth:	12/6/1989	**Litigation:**	No **Direct Access:** No

#	Question	Answer	Value	Scale
	Pain Level	0	**0**	(0 to 10)
	Pain Intensity	The pain is very mild at the moment.	**1**	(0 to 5)
	Personal Care (washing, dressing, etc.)	I can look after myself normally but it causes extra pain.	**1**	(0 to 5)
	Lifting	I can lift heavy weights but it gives me extra pain.	**1**	(0 to 5)
	Headache	I have moderate headaches which come infrequently.	**2**	(0 to 5)
	Recreation	I am able to engage in my recreational activities with some pain	**1**	(0 to 5)
	Reading	I can read as much as I want with slight neck pain.	**1**	(0 to 5)
	Work	I can only do my usual work but no more.	**1**	(0 to 5)
	Sleeping	My sleep is slightly disturbed (< 1 hr sleep loss).	**1**	(0 to 5)
	Concentration	I can concentrate fully when I want with no difficulty.	**0**	(0 to 5)
	Driving	I can drive my car without neck pain.	**0**	(0 to 5)

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Rehab Solutions
Advanced Physical Therapy

Missed Appointment

Patient Name: PULLIAM, VIKTORIYA (Discharged)
Date of Birth: 12/06/1989
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified

Date of Cancel: 02/14/2022
Physician Name: MANSELL, JOHN MD

Reason for missed appointment: Cancel
HAS STUFF GOING ON WITH KIDS AT SCHOOL FOR VALENTINES DAY. ST

Shauna Tranas
License #265546
Document created on February 14, 2022 at 5:31 pm

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/04/2022

Pt to f/u in 2 weeks, to perform HEP and return to exercise. Pt encouraged to f/u sooner if needed, especially if sleep is affected by sx.

Testing:
- OH, BP w/ valsalva, consider treadmill test w/ BP
- Vertebral artery positional testing

Tx:
- scar mobilization
- upper cervial mobilization, STM/MFR
- thoracic mob/manip

Dylan Ziehme

Dylan Ziehme
Initiated by Dylan Ziehme on March 4, 2022 at 10:26 am

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Electronically Co-Signed by Aaron Proefrock on March 4, 2022 at 12:14 pm

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/04/2022

GP:97140 Manual Therapy 3

Reassessment of ROM and scar mobility
STM/MFR to suboccipitals, scalenes, paraspinals, R>L
C1-3 CPA/UPAs with side bend and rotation, supine, grade 1-4
Passive stretching into side bend and rotation
scar massage x 10 mins including prolonged stretches and skin rolling

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Objective Findings

Treadmill BP testing - walking steady speed @ 2.7 mph
At rest: 138/98
2 min: 138/98
4 min: 136/98
Immediately following 10 min walking, sitting: 150/96
After 1 min rest, sitting: 130/90

Assessment

Assessment/Diagnosis: The patient returns to therapy for tx s/p cervical fusion in response to 2 MVAs. She complains of 1. hypertrophic/keloid scar appearance and mobility, 2. new headaches, which seem to happen after every workout, and 3. new hypertension which occurs early in the morning and later in the evening. Pt reports no significant changes in sx after initial evaluation, but is no worse.

She returns today after a 1-week vacation. She continued to have neck pain and headaches, but is not sure if this is from her condition or sleeping in a hotel and drinking alcohol. She is back to her usual routines and will start exercising again Monday. She will monitor her sx and response to tx over the next few weeks.

Currently, the DDX for the pt's headaches and exertional symptoms includes:
1. Whiplash Associated Disorder
2. mTBI with features of autonomic dysfunction
3. Headache (By ICDH-3 criteria):
- 4.2.1 Probable primary exercise headache
- 4 Other primary headaches
- 10.3 Headache attributed to arterial hypertension
- 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy

We will continue to investigate the hypertension and exercise related headaches further with orthostatic and stress testing. The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits, scar remodeling and sensitivity, neck pain, and to return to PLOF.

Patient Education: Continue scar massage and use of silicone. Instruction in neck stretches for HEP. Instruction in desensitization of scar.

Rehab Potential: Good

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP |
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep. |

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within |
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM. |
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck. |

Plan

Instructions: Progressing Patient Next Visit

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Daily Note / Billing Sheet

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Referring Physician(s): ACCESS, DIRECT MD

Date of Daily Note: 03/04/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Surgery: (Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 2

Date of Original Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Insurance Name:

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
Current Complaints / Gains: Pt returns to PT following a one-week vacation in Florida with family. She continues to have neck pain and headaches. Today pt would like to focus on scar care and neck pain.
Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Home Health Care: No
History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Mental Status/Cognitive Function Appears Impaired? No

Objective

CPT® Code	Direct Timed Codes	Units
GP:97110	Therapeutic Exercise BP testing response to walking on treadmill, comfortable pace, x 10 min HEP prescription: - scalene stretch - look into armpit stretch - chin tuck + flexion stretch	1

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/18/2022

Plan

Instructions: Progressing Patient Next Visit

Testing:
- OH, BP w/ valsalva, consider treadmill test w/ BP
- Vertebral artery positional testing

Tx:
- scar mobilization
- upper cervial mobilization, STM/MFR
- thoracic mob/manip
- lumbar stiffness and pain

Dylan Ziehme

Dylan Ziehme
Initiated by Dylan Ziehme on March 28, 2022 at 8:26 pm

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Electronically Co-Signed by Aaron Proefrock on March 29, 2022 at 7:21 am

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/18/2022

scar mobilization including longitudinal, perpendicular, rotation, prolonged stretch, and skin rolling over scar and surrounding areas.
B Cervical spine mobilizations grade 3-4 - IMP/SAL, upper cervical UPA/rotations
sub occipital release
STM/MFR to B traps, cervical paraspinals, scalenes, SCMs
HVLA prone PA thrust mobilization to mid thoracic spine. Pt has no contraindications to thrust techniques; tolerated well.

GP:97530 Therapeutic Activity/Kinetic 1

BP readings for OH - negative, but 12 mmHg drop in systolic and 6 mmHg drop in diastolic.
HEP review and hydrogel instruction

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Assessment

Assessment/Diagnosis: Vicki's headaches seem to be responding to manual therapy techniques. We will introduce proprioception and endurance training in upcoming visits.
She was able to order gel for her scar. Lumbar ROM is stiff and limited into multiple planes. Plan to assess in greater detail next session.

Currently, the DDX for the pt's headaches and exertional symptoms includes: 1. Whiplash Associated Disorder 2. mTBI with features of autonomic dysfunction 3. Headache (By ICDH-3 criteria): - 4.2.1 Probable primary exercise headache - 4 Other primary headaches - 10.3 Headache attributed to arterial hypertension - 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy We will continue to investigate the hypertension and exercise related headaches further with orthostatic and stress testing. The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits, scar remodeling and sensitivity, neck pain, and to return to PLOF.

Rehab Potential: Good

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP |
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep. |

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within |
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM. |
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck. |

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Daily Note / Billing Sheet

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Referring Physician(s): MANSELL, JOHN MD

Surgery: (Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 3

Insurance Name:

Date of Daily Note: 03/18/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified
Date of Original Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
Current Complaints / Gains: Vicki reports not ordering the hydrogel for her scar. Decided to order on her phone in clinic while she was warming up. Her spine has been sore and thinks that the nerve ablation has worn off and she might need to get a repeat procedure.
Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Home Health Care: No
History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Mental Status/Cognitive Function Appears Impaired? No

Objective

CPT® Code	Direct Timed Codes	Units
GP:97110	Therapeutic Exercise AROM and movement assessment of lumbar spine	1
GP:97140	Manual Therapy	2

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Phone: (307)686-8177
Fax: (307)686-9484

Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/21/2022

Testing:
- OH, BP w/ valsalva, consider treadmill test w/ BP
- Vertebral artery positional testing

Tx:
- scar mobilization
- upper cervial mobilization, STM/MFR
- thoracic mob/manip
- lumbar stiffness and pain

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Electronically Signed by Aaron Proefrock on March 21, 2022 at 6:10 pm

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/21/2022

ROM baselines for Lumbar Spine: Pain/ stiffness with SG and Ext.

Sustained Prone
Sustained Prone on elbows
Press ups x10
Press ups with HOS L 3x10 -- greater ROM, Produced, NW

REPEX: x25 reps, 2* flex, 10* ext

amb trials and AROM rechecks

GP:97140 Manual Therapy 1

Cupping x6 to lumbar spine and along iliac crests
PA pressure and assessment of SI and Lower lumbar segments

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Assessment

Assessment/Diagnosis: Vicki shows improved ROM with some pain while performing REL HOS L. She did not respond well to the REPEX as she was stiff afterwards and had difficulty with flexing while re-testing baseline AROM. Fair response to cupping. Plan to assess response to determine appropriate HEP for lumbar pain and stiffness.

She will monitor her sx and response to tx over the next few weeks. Currently, the DDX for the pt's headaches and exertional symptoms includes: 1. Whiplash Associated Disorder 2. mTBI with features of autonomic dysfunction 3. Headache (By ICDH-3 criteria): - 4.2.1 Probable primary exercise headache - 4 Other primary headaches - 10.3 Headache attributed to arterial hypertension - 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy We will continue to investigate the hypertension and exercise related headaches further with orthostatic and stress testing. The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits, scar remodeling and sensitivity, neck pain, and to return to PLOF.

Patient Education: Continue scar massage, use of silicone, and neck stretches for HEP.
Monitor response to REL with HOS and then try some prior to next visit in 5 days.

Rehab Potential: Good

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP |
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep. |

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within |
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM. |
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck. |

Plan

Instructions: Progressing Patient Next Visit

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Daily Note / Billing Sheet

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Referring Physician(s): ACCESS, DIRECT MD

Surgery: (Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 3

Insurance Name:

Date of Daily Note: 03/21/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus
Date of Original Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
Current Complaints / Gains: Pt did not have too much pain after testing her lumbar spine last time. She is hoping to work on lower back pain today.
Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Home Health Care: No
History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Mental Status/Cognitive Function Appears Impaired? No

Objective

CPT® Code	Direct Timed Codes	Units
GP:97110	Therapeutic Exercise	3

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Fax: (307)686-9484

Physical Therapy Initial Examination

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

Plan

Frequency: 1-2 visits per week for 2-3 weeks, then prn
Duration: 12 weeks
Plan: Begin Plan as Outlined
Treatment to be provided:

Procedures

Therapeutic Exercises, Therapeutic Activity, Neuromuscular Rehabilitation, Manual Therapy, Massage, Vestibular Rehabilitation, Patient Education, Self Care

Modalities

To Improve (Pain Relief, Decrease Inflammation, Increase Blood Flow, Improve Tissue Healing)

Certification of Medical Necessity: It will be understood that the treatment plan mentioned above is certified medically necessary by the documenting therapist and referring physician mentioned in this report. Unless the physician indicates otherwise through written correspondence with our office, all further referrals will act as certification of medical necessity on the treatment plan indicated above.

Thank you for this referral. If you have questions regarding this plan of care, please contact me at (307)686-8177.

Thanks!

Dylan Ziehme

Dylan Ziehme
Initiated by Dylan Ziehme on February 8, 2022 at 8:45 am

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Electronically Co-Signed by Aaron Proefrock on February 8, 2022 at 9:26 am

Please sign and return: Fax#: (307)686-9484

I certify the need for these services furnished under this plan of treatment and while under my care.

X I have no revisions to the plan of care.
__Revise the plan of care as follows_____

Physician Signature_____
D. ACCESS, MD
Date:_____ Time:_____

Electronically signed by JOHN L. MANSELL, MD on 03/25/2022 at 08:18::20 AM

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Physical Therapy Initial Examination

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

Assessment/Diagnosis: The patient presents for my initial evaluation s/p cervical fusion in response to 2 MVAs. She complains of 1. hypertrophic/keloid scar appearance and mobility, 2. new headaches, which seem to happen after every workout, and 3. new hypertension which occurs early in the morning and later in the evening.

The patient presents with cervical ROM limitations consistent with her cervical fusion, a hypertrophic scar with hypersensitivity and limited mobility, hypertension of 140/60 in the clinic today, and joint and soft tissue irritation throughout the cervical spine.

Physical therapy intervention will be able to address the limitations in ROM and scar mobility and appearance. Physical therapy intervention is also likely able to address the patient's headaches and hypertension, but further assessment is required to confirm. Additionally, the therapy team will collaborate with the patient's medical providers as appropriate.

We will continue to investigate the patient's headaches. Currently, the DDX includes:

1. Whiplash Associated Disorder
2. mTBI with features of autonomic dysfunction
3. Headache (By ICDH-3 criteria):
- 4.2.1 Probable primary exercise headache
- 4 Other primary headaches
- 10.3 Headache attributed to arterial hypertension
- 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy

We will also investigate the hypertension further with orthostatic and stress testing.

The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits and scar sensitivity.

Patient Clinical Presentation: The clinical presentation is evolving with changing characteristics.

Patient Education: Instruction in scar massage. Advised to continue use of silicone scar patches and moisturize with a gentle moisturizer.

Following the evaluation and extensive patient education regarding diagnosis, prognosis, and treatment goals, the patient (parent/guardian, power of attorney holder) actively participated in the creation of the current goals and agrees to the current treatment plan.

Rehab Potential: Good

Contraindications to Therapy: None

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep.

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM.
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck.

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Physical Therapy Initial Examination

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

No Upper Extremity strength deficits were noted.

Neuro-Vascular

Complaints of any radicular symptoms in either extremity

Complaints of any radicular symptoms in either extremity	No

Myotomes Upper

	Right	Left
C3 Lat Cerv Flexion	Not Tested	Normal
C4 Shoulder Elevation	Good	Good
C7 Triceps	Good	Good
C8 1st Extension	Good	Good
T1 Hand Intrinsics	Good	Good

Special Tests

Comments CRLF equal without limitation

Palpation

Palpation
Palpable tenderness or increased muscular tone noted.

Right SCM	Normal
Left SCM	Normal
Right Scalenes	Tender with increased tissue tension
Left Scalenes	Tender with increased tissue tension
Right Suboccipital Myofascia	Normal
Left Suboccipital Myofascia	Normal
Right Cervical Paraspinals	Normal
Left Cervical Paraspinals	Normal
Right Levator Scapula	Normal
Left Levator Scapula	Normal
Right Upper Trapezius	Normal
Left Upper Trapezius	Normal

Assessment

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Physical Therapy Initial Examination

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 02/07/2022

History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Medical History Review: The patient has a history of present problem with a history of 1-2 personal factors and/or comorbidities that impact the plan of care.
Mental Status/Cognitive Function Appears Impaired? No
Current Medications: Prescription (escitalopram 20mg, triptan drug for migraines), Over The Counter (tylenol), Other (topical neosporin pain relief)
Patient Goals: improve scar mobility and appearance

Objective

Inspection

Patient Consent

Patient/Parent/Guardian Consent	Yes
Inspection	incision well-healed; keloid/hypertrophic scar present with mildly impaired mobility. Well-appearing.

Outcome Measurement Tools

Spine
Neck Disability Index Questionnaire

Total Score:	9
Total %:	18%

Observation

Vitals

Blood Pressure	Left Arm: 140/60, Upper Arm, Sitting

Movement Loss Cervical

Flexion	Minor
Extension	Minor

Range of Motion

Cervical AROM

Forward Bending	1 fist from chest
Backward Bending	15%
Right Rotation	WFL
Left Rotation	WFL
Right Side Bending	50-60%
Left Side Bending	50-60%
Cervical AROM Comments	minor restrictions all directions consistent with cervical fusion.

Strength

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Physical Therapy Initial Examination

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989

Referring Physician(s): ACCESS, DIRECT MD

Surgery: (Date/Type) 11/04/2021 cervical fusion 3 levels
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Date of Initial Examination: 02/07/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury, Feb 2020 2nd MVA
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus
Visit No.: 1

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
History of Present Condition/Mechanism of Injury: Cervical fusion and scar:
Following 2 MVAs (2019 and 2020), pt experienced unremitting neck pain with radicular s/sx, which was treated with a three-level cervical fusion with anterior approach.

Headaches:
Pt has an existing dx of migraines without aura. She takes a triptan as an abortive medication. She complains of new headaches over the past 2 weeks that occur after exercise. Her triptan does work on these headaches.

Hypertension:
Pt reports feeling her blood pressure go up recently, so she has been measuring it at home. She reports readings near 175/106, especially in the early morning or late evening. Her typical BP prior to this injury was normal, near 120/60-80.

Social / recreation:
Administrative work, on laptop. Works from home, moves around house with laptop.
Exercise: goes to gym: yoga, metcon, spin. some tightness in neck after workouts, this is getting better.
Primary Concern/Chief Complaint: CC: scar care
Secondary complaints: New HAs, new hypertension

Patient presents to therapy primarily to address the scar from her anterior cervical fusion. She also has a two week history of new headaches, which feel like her existing migraines, but occur after exercise. She also complains of new hypertension, especially in the morning and at night. Her upper cervical spine bothers her while trying to fall asleep since surgery.
Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Pain Location: neck over scar
Pain Scale: Worst: 3 **Best:** 0 **Current:** 0
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
General Health:
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Is Patient A Tobacco User: No
Home Health Care: No

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Daily Note / Billing Sheet

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Document Date: 03/25/2022

Assessment

Assessment/Diagnosis: Vicki reports very good response to cupping for her low back last visit and her back feels like it is improving. She reports improvement in her neck as well but is still bothered by her scar. Restrictions are felt in multiple directions over the scar.

Currently, the DDX for the pt's headaches and exertional symptoms includes: 1. Whiplash Associated Disorder 2. mTBI with features of autonomic dysfunction 3. Headache (By ICDH-3 criteria): - 4.2.1 Probable primary exercise headache - 4 Other primary headaches - 10.3 Headache attributed to arterial hypertension - 10.3.2 Headache attributed to hypertensive crisis without hypertensive encephalopathy We will continue to investigate the hypertension and exercise related headaches further with orthostatic and stress testing. The patient requires physical therapy services to investigate her headaches and hypertension further, as well as to treat her mobility deficits, scar remodeling and sensitivity, neck pain, and to return to PLOF.

Patient Education: Continue scar massage, use of silicone, and neck exercises for HEP. Discussed potential use of TENS for period pain - pt will ask her OB if this is a good option for her regarding fertility. Risks of use over metal implants were discussed. Pt was also advised that we offer pelvic rehab if desired, and that we can teach her to use a TENS unit if she would like.

Rehab Potential: Good

Patient Problems:
- New headaches
- Pre-existing migraines
- hypertrophic scar with limited mobility
- limited cervical ROM s/p fusion
- new hypertension

Short Term Goals:
1: (2 Weeks) | Patient will be compliant with HEP |
2: (6 Weeks) | Pt will report resolution of her upper cervical spine pain while falling asleep. |

Long Term Goals:
1: (12 Weeks) | Patient will return to PLOF pain free within |
2: (10 Weeks) | Patient will improve scar mobility so that she does not feel limitation of motion or cavitation with ROM. |
3: (8 Weeks) | Patient will demonstrate reduced scar sensitivity to tolerate a variety of clothing touching her neck. |

Plan

Instructions: Progressing Patient Next Visit

Testing:
- OH, BP w/ valsalva, consider treadmill test w/ BP
- Vertebral artery positional testing

Tx:
- scar mobilization
- upper cervial mobilization, STM/MFR
- thoracic mob/manip
- lumbar stiffness and pain

Dylan Ziehme

Dylan Ziehme
Initiated by Dylan Ziehme on March 25, 2022 at 5:43 pm

Andrew Nielsen, PT, DPT, CSCS
License #1657
Electronically Co-Signed by Andrew Nielsen, PT, DPT, CSCS on March 28, 2022 at 11:55 am

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Daily Note / Billing Sheet

Rehab Solutions
Advanced Physical Therapy

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Referring Physician(s): MANSELL, JOHN MD

Surgery: (Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 4

Insurance Name:

Date of Daily Note: 03/25/2022
Injury/Onset/Change of Status Date: 11/23/2019 New Injury
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus
Date of Original Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

Subjective

Movement-Based Spinal Assessment Questionnaire
Work: Mechanical Stresses: laptop use 100%, from home
Commenced as Result of: MVA
Symptoms at Onset: Neck, Arm; numbness and tingling
Current Complaints / Gains: Pt had horrible period pain for the last 3 days so she has not been able to perform HEP. Now that she is feeling better, her back and neck are doing OK. She reports her back is improving and that cupping was helpful. She also reports her BP has still been fluctuating. She sees her surgical team on April 5.
Before the injury/onset/change of status date, the patient was able to perform the following activities:
Self Care:
Changing & Maintaining Body Position:
Mobility: Walking & Moving Around:
Carrying, Moving & Handling Objects:
Current Functional Limitations:
Other: limited motion of surgical scar and limited motion of cervical spine d/t fusion.
Aggravating Factors: clothes touching scar, prolonged immobility (sleeping)
Occupation/Social History:
Social History: Married
Name of Occupation: Administrator
Status of Occupation: Full Time
Duty Level: Sedentary
Description: laptop use, from home
Workers Compensation: No
Home Health Care: No
History of Falls: Yes; ; 1 fall - after surgery, related to running out of meds which made her dizzy/unsteady, then slipped on ice.
Falls Documented: No
Medical History: Headaches - new
Migraines - existing
2 MVAs
pituitary tumor present.
low thyroid hormone
unable to conceive over last few years.

Complicating/Personal Factors: Mechanism of injury/ Illness, Previous Therapy (chiropractic), Psycho-Social (Anxiety,), Surgical History
Mental Status/Cognitive Function Appears Impaired? No

Objective

CPT® Code	Direct Timed Codes	Units
GP:97140	Manual Therapy scar mobilization including longitudinal, perpendicular, rotation, prolonged stretch, and skin rolling over scar and surrounding areas.	3

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Rehab Solutions
Advanced Physical Therapy

Missed Appointment

Patient Name: PULLIAM, VIKTORIYA (Discharged)
Date of Birth: 12/06/1989
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified

Date of Cancel: 04/01/2022
Physician Name: MANSELL, JOHN MD

Reason for missed appointment: Cancel
WANTS TO WAIT UNTIL AFTER SHE SEES HER DOCTOR. ST

Chanel McLain
License #337741
Document created on April 1, 2022 at 3:38 pm

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Rehab Solutions
Advanced Physical Therapy

Call to Pt

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified

Date of Case Note: 05/23/2022
Physician Name: MANSELL, JOHN MD

Called and LMOM. How is pt. doing? Does she wish to schedule an appointment or still hold off until after she gets tx for lumbar and cervical spine?

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Document created on May 23, 2022 at 10:47 pm

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Rehab Solutions
Advanced Physical Therapy

Discharge Note

Patient Name: PULLIAM, VIKTORIYA
Date of Birth: 12/06/1989
Physician Name: J. MANSELL, MD

Surgery:(Date/Type) 11/04/2021 cervical fusion 3-levels
Visit No.: 5

Date of Discharge Note: 06/10/2022
Injury/Onset Date: New Injury, Feb 2020 2nd MVA
Diagnosis: ICD10: M43.22: Fusion of spine, cervical region, L91.0: Hypertrophic scar, R51.9: Headache, unspecified, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus, M54.50: Low back pain, unspecified
Date of Last Eval: 02/07/2022
Treatment Diagnosis: ICD10: L91.0: Hypertrophic scar, R51.9: Headache, unspecified, M43.22: Fusion of spine, cervical region, R03.0: Elevated blood-pressure reading, without diagnosis of hypertension, G43.009: Migraine without aura, not intractable, without status migrainosus

PULLIAM, VIKTORIYA has been discharged from our care for the following reasons:
Pt. has self discharged at this time.

Please see the last progress report or notes above for the patient's most recent status. Thank you for the opportunity to assist you in the rehabilitation of this patient. If you have any questions or concerns, please feel free to contact me at (307)686-8177.

Aaron Proefrock, PT, DPT

Aaron Proefrock
License #PT-2016
Document created on June 10, 2022 10:15 am

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212 Degrees LLC

Itemization of Charges

For Posting Date June 8, 2023

Clinic:	212 Degrees LLC		
Address:	1103 E Boxelder Rd Ste U Gillette, WY 827185582	**Insurance #1:**	AETNA-Commercial 123 MAIN ST WESTCHESTER, IL 60154
Phone:	(307) 686-8177	**Adjuster:**	
Tax ID:	261286085	**Group Number**	0699924-013-00001
WCAB:		**Policy Number:**	W245911436
Employer:		**Insurance #2:**	
Patient #:	45032	**Adjuster:**	
Patient:	VIKTORIYA PULLIAM	**Group Number:**	
Date of Injury:	11/23/2019	**Policy Number:**	

Visit #	Service Date	Provider Name	Procedure Description	Code	Charges	Adjust	Payments	Balance
133501	02/07/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ma...	97140	140.00			-
133501	02/07/2022	Aaron Proefrock, PT, DPT	Evaluation for physical th...	97162	178.00			-
137044	03/04/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ex...	97110	63.00			-
137044	03/04/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ma...	97140	210.00			-
140940	03/18/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ex...	97110	63.00			-
140940	03/18/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ma...	97140	140.00			-
140940	03/18/2022	Aaron Proefrock, PT, DPT	Therapy procedure using fu...	97530	70.00			-
139928	03/21/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ex...	97110	189.00			-
139928	03/21/2022	Aaron Proefrock, PT, DPT	Therapy procedure using ma...	97140	70.00			-
140836	03/25/2022	Andrew Nielsen, PT,DPT	Therapy procedure using ma...	97140	210.00			-
TOTALS					**1,333.00**			
							BALANCE DUE	-